Position Paper

<u>Concordion IV Conference</u>

Committee: World Health Organization (WHO) | Agenda: The question of the implementation and usage of palliative instead of curative medication | Delegation: Belgium | Name: Jules Kitto-Astrop

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The question of the implementation and usage of palliative instead of curative medication remains a highly controversial topic amongst medical discussions, with Belgium taking a compassionate and human-rights based approach. Belgium believes that when recovery is no longer possible, the focus of care should be on ensuring the best quality of life before death.

The World Health Organization (WHO) encourages the use and integration of palliative care worldwide. In 1997, Belgium developed palliative care networks, including home teams and care centers. Since then care has been further implemented and developed with the development of mobile palliative care teams that may control and direct the care. Belgium, like the WHO, stands firmly with the belief that palliative care should be integrated into general care and should be accepted upon reasonable request. Belgium highlights the consideration of treatment resistant mental illness as an example of palliative and curative integrated care, with many disorders having no specific cure, medication is often prescribed for symptom management, a prime example of palliative care, though this may result in more extreme measures such as euthanasia when options have been exhausted. Belgium states that palliative care is reserved for individuals with little to no hope of recovery, or are at the end of life. Despite this, statistics show that 1 in 5 terminal cancer patients died within a week of receiving palliative care (848, “End of Life Care”), because it is a last resort and is only implemented at the very end of life, though this is debatably not compassionate. On the other hand, these measures require certain medications that are highly addictive and dangerous regulated substances, the access to these drugs is extremely limited making it hard to receive the right palliative care. The WHO calls for the liberalization of access to essential opioid pain relief for palliative care purposes, as such Belgium fails to highlight this with delayed palliative care. Furthermore, the economic burdens that are a result of the expensive but necessary care given to terminal patients, are highly debated with the ultimate question being “How much is willing to be spent on someone's last month alive?”, aiming to improve quality of life is empathetic however must be balanced with the economic factors. Cost analyses show that the costs of palliative care are actually lower than the costs of usual care (Simoensa et al.) yet still high (estimated at €2,456) indicating a need for systematic changes to better consider economic limitations and social status in the face of terminal care.

Above all, it is important to maintain autonomy and choice. Belgium urges all nations to assess this dire need for palliative care as it is a fundamental human right and a standard of compassion when one is at the end of life. Belgium acknowledges the increasing number of opioid abuse cases which indicates caution is necessary when prescribing such addictive medications, yet Belgium follows WHOs “smart regulation” models for the liberalization of opioid medications. Lastly, Belgium urges all nations to standardize regulations around palliative care and utilise technology to promote the best path of care for patients.

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The question of the legalization and regulation of euthanasia is one that Belgium stands at the forefront of, being the second nation to legalize euthanasia, Belgium strongly advocates for individual choice, autonomy, and dignified death through ethical euthanasia. Belgium believes when one is suffering immensely, the most compassionate thing to do is to allow them a way out.

The World Health Organization (WHO) has remained neutral on the topic of euthanasia showing the extent of the divide in the discussion. In September of 2002, Belgium legalized euthanasia for those experiencing constant and unbearable physical or mental suffering that cannot be alleviated. Upon implementation Belgium recieved roughly 20-30 cases per month indicating this decision was likely long overdue (KCE). Belgium stands with the belief that euthanasia when carried out ethically, can be the best and only option. Despite this, Belgium also remains sensitive to the weight carried by medical professionals in relation to euthanasia, stating that "*physicians cannot be compelled to perform euthanasia and other persons cannot be forced to assist in it.*" (Article 14, Belgian Act on Euthanasia). Therefore procedures exist to ensure due process and safety of patients and medical professionals through referrals that occur upon request. In Belgium and worldwide, euthanasia carries low levels of support however executive decisions to legalize euthanasia ensure that autonomy is given, though many argue Belgium is an example of a "slippery slope" with such legislations. In 2012, Tom Mortier was informed that his mother, at the age of 64, was euthanized the day prior due to her treatment resistant depression. This is an example of improper practices, in all situations any available family members should be contacted to ensure the choice is made with all factors in mind. Furthermore, some call for the separation of the procedure from the practice of medicine however Belgium remains firm that euthanasia must be embedded in a palliative total care approach. Belgium acknowledges the high demand for euthanasia and intends to reduce this through the integration of palliative care which may prevent euthanasia through the improved quality of life.

Finally, Belgium calls for stricter global policies around euthanasia but remains firm in the legalization of the procedure. Ethical euthanasia must occur only when all other options have been exhausted and palliative care has been implemented to the best of its ability. Belgium urges nations to ensure no procedure may occur without the knowledge of immediate family and the complete consent of all participating parties, especially the involved medical professionals. Euthanasia must be seen as a last resort not as a simple solution, waiting periods must be better implemented and screenings must be done by several medical professionals to confirm the necessity of this irreversible option.

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